HAIR LOSS
01

Finasteride

Synthetic 4-azasteroid that blocks type II 5α-reductase, taken by mouth



CLASS
5α-reductase inhibitor

STUDIED FOR
Hair loss, enlarged prostate

HUMAN EVIDENCE
Large trials; FDA approved

US STATUS
FDA approved (Propecia, Proscar)

STUDIED FOR

Male pattern loss	Enlarged prostate	Topical spray
HUMAN	HUMAN	HUMAN

Not indicated for women. A 12-month trial in 137 postmenopausal women showed no improvement over placebo. No topical form is FDA approved.

HOW IT WORKS IN THE BODY

Finasteride → 5α-reductase II → Less DHT → Scalp, prostate

- **Competitive inhibitor** of type II 5α-reductase, per the label
- **About 100-fold selective** for type II over type I, in lab binding studies
- **Blood DHT** fell about 65% within 24 hours of a 1 mg tablet
- **Testosterone** rose about 15%, staying in the normal range

EVIDENCE & WHAT'S KNOWN

EVIDENCE

FDA APPROVED
LEVEL 5 OF 5

- Approved for hair loss in 1997
- Three 12-month trials, 1,879 men
- Blinded extensions up to 5 years

NOT YET ESTABLISHED

- Rate of lasting sexual effects
- Role of each DHT drop
- Compounded topical safety

QUICK PRACTICAL INFO

HALF-LIFE
About 5 to 6 hours (men 18 to 60); 8 hours over 70

ROUTES STUDIED
By mouth (tablets)

PHARMACOKINETICS
Peak in 1 to 2 hours; liver metabolism (CYP3A4)

SOLD AS
Prescription tablets (Propecia, Proscar)

MONITORED IN TRIALS
PSA (lowered), sexual effects, mood

SPORT (WADA)
Not on the 2026 list

REGULATORY

FDA	Approved (1992, 1997); no topical form approved
EMA	2025 review added suicidal ideation (tablets)
MHRA	Patient card on psychiatric, sexual effects
WADA	Not on the 2026 list

REFERENCES

1. FDA, Propecia prescribing information (DailyMed), Oct 2024
2. Kaufman et al., J Am Acad Dermatol, finasteride in male pattern hair loss, Oct 1998
3. EMA, finasteride and dutasteride referral, 2025

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